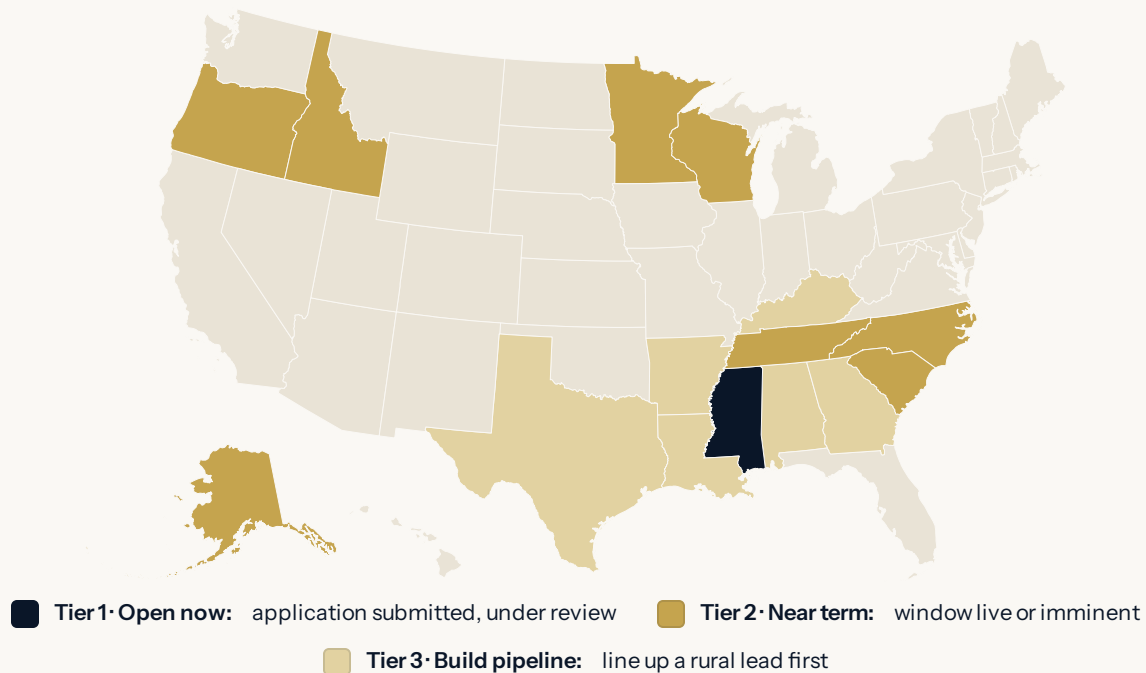


Submitted in Mississippi. Built to repeat everywhere.

The program is live in all 50 states and our first application is already in: submitted with Honeycomb in Mississippi. Our targets are the states where an open technology window meets a rural provider we can apply through.

AUDIENCE **Internal** • **Investors** VERSION **[260805.2]** STATUS **Confidential Draft**

\$50 billion is moving to the states over five years, first-year awards averaged about \$200 million each (a \$147M–\$281M range), and every dollar must be obligated by October 30, 2026. That deadline pressure is pushing money to providers now, but states are moving at very different speeds: some are taking applications today, others are still negotiating their CMS agreements. Targeting is about timing and access, not just size.



State stages shift weekly; each opportunity is verified against the state's own posting before we act.

HOW WE CHOOSE

Four factors set the order: an **open window**, the **right program** (technology, not workforce-only), **provider access** (a rural in-state applicant or a direct-vendor lane), and **rural density**.

Open now, near term, and the pipeline to build.

TIER 1 • SUBMITTED • JULY 2026 • UNDER STATE REVIEW

Mississippi • Rural Provider Technology Grant

\$300K

TOTAL SUBMITTED ASK, TWO LANES

\$165K

TECHNOLOGY LANE (RTG)

\$135K

TELEHEALTH LANE (TAPS)

Honeycomb Medical Group's rural clinics in north Mississippi (Holly Springs, Marshall County) made it a directly eligible applicant, with InPursuit as the named technology partner. **The application was submitted in July 2026 and is under state review:** \$165K for the technology backbone through the Rural Provider Technology Grant, inside the state's \$200K–\$400K per-award guidance, and \$135K for vitals-verified telehealth through the separate TAPS pool. Mississippi's total first-year award is \$205.9M. Figures are the amounts as submitted; award decisions rest with the state. First revenue and the repeatable proof start here.

TIER 2 NEAR TERM	North Carolina \$213.0M • HOME STATE Tennessee \$206.9M Wisconsin \$203.7M Minnesota \$193.1M South Carolina \$200.0M Oregon \$197.3M Idaho \$186.0M Alaska \$272.2M Technology and health-IT grants live or imminent, each needing a rural in-state applicant. North Carolina leads, home state, the nation's second-largest rural population, and a plan built on HIE, AI, and value-based payment; access runs through its regional ROOTS Hubs. Tennessee's windows are strong but Honeycomb's Memphis base isn't rural; Minnesota buys vendor services directly.
TIER 3 BUILD PIPELINE	Arkansas \$208.8M Alabama \$203.4M Louisiana \$208.4M Kentucky \$212.9M Georgia \$218.9M Texas \$281.3M The deeply rural cluster around Honeycomb's Mississippi footprint, plus Texas, the largest rural prize, open to tech vendors via state registration. Windows are still forming; the work is sourcing in-state rural leads.

Figures are each state's **total** first-year RHTP award; the technology slice we target is a subset (in Mississippi, the RTG pool is \$5M at \$200K–\$400K per award).

THE PLAY

Convert the submitted application, work the near-term windows, build the pipeline.

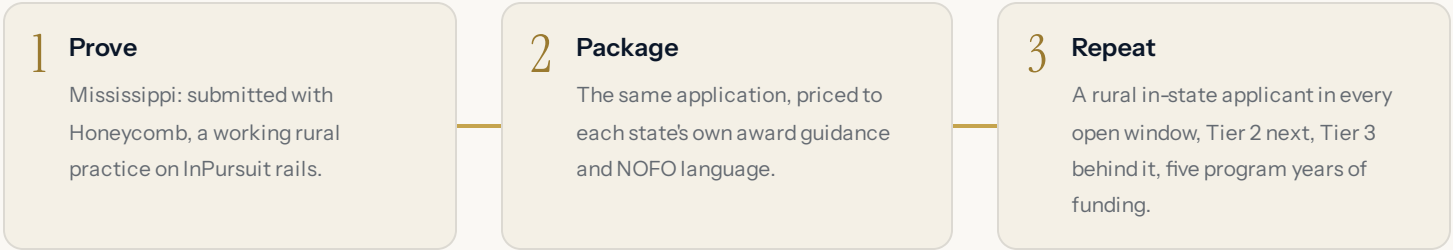
Mississippi is in and under review: proof and first revenue. In near-term states, pair with a rural in-state applicant, or bid directly where a state buys technology itself. In pipeline states, source in-state providers ahead of the windows. The constant: InPursuit is the named technology, a rural in-state provider is the applicant, and timing is everything.

WHY THIS REPEATS

One infrastructure, every state, the same play.

Rural providers do not lack demand for technology; they lack the capital, the IT staff, and the integration lift to adopt it. RHTP removes the capital barrier. The InPursuit Orchestration Layer removes the rest: one integration with the practice's existing EMR carries the complete record, the health-information-exchange connection, the analytics, remote monitoring, and vitals-verified telehealth, delivered as infrastructure the practice holds, not per-seat licenses that outlive the award.

THE REPEATABLE CYCLE



WHAT PROVIDERS NEED

Capability without the burden

Grant dollars buy the technology once at a fixed price; the care it enables bills normally. Whoever does the work bills for the work, and no fee ever passes from a practice to InPursuit for care delivery. Every competitor sends the practice an invoice; this model sends the practice capability.

WHAT COMMUNITIES NEED

Care that actually reaches them

Patients carry their complete record in the InPursuit Health App at no grant cost. Telehealth visits capture FDA-cleared vitals by face and finger scan on the patient's own phone, no peripherals to ship, charge, or lose across rural distances, and chronic conditions are monitored between visits instead of waiting for the next drive to town.

WHY REVIEWERS SCORE IT

A system, not a shopping list

State reviewers read hundreds of isolated purchases. This is one integrated infrastructure meeting the interoperability, security, and sustainability standards both grant lanes demand, with independent credentials behind it: CMS Health Tech Ecosystem General Availability across all three use cases, and the DiMe Seal, awarded July 14, 2026, valid through July 13, 2027.

WHY IT COMPOUNDS

Rails sized for the next practice

Each award deploys infrastructure sized so additional practices in that state onboard onto the same rails in program years two through five. The first application in a state is the expensive one; every one after it is the playbook running again.

Sources. CMS RHT Program announcement (cms.gov; HHS, Dec 2025), \$50B over FY2026–2030, awards \$147M–\$281M. State award amounts: state RHTP agencies (MS, TN, MN, OR, ID, SC, AK), CSG South FY26 table, KFF/Aurrera Health. Rollout: NRHA tracker (ruralhealth.us); Telehealth.org (Jun 2026). Figures change; verified against each state's posting before action. Submitted application figures (\$300K total: \$165K RTG, \$135K TAPS): InPursuit and Honeycomb Medical Group joint submission, July 2026; award decisions rest with the State of Mississippi. No InPursuit projections appear here.